BASELINE → TREATMENT → FOLLOW-UP

When checks happen

The columns show order, not equal periods. There is no fixed number of cycles for these plans. Agree who books each check, when results are reviewed and whom to contact if a check is missed.

Check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Blood count, kidney and liver	Baseline blood count, kidney and liver profile	Review results and symptoms before treatment	Blood count, kidney and liver profile before every cycle; weekly blood counts when delayed for low counts	Repeat sooner if unwell or results need follow-up	Follow unresolved abnormalities after treatment
Magnesium, calcium and potassium	Check mineral levels before starting	Repeat periodically during treatment	Keep blood-test and replacement appointments	More checks may be needed if levels fall or diarrhoea develops	Monthly checks for two months after the last antibody dose
Skin, nails, eyes and breathing	Discuss existing skin, eye or lung problems	Start the prescribed skin-care plan; report reactions early	Review skin, nails, diarrhoea and new symptoms	Eye pain, worsening breathing or severe rash needs prompt assessment	Continue care until symptoms and mineral levels settle
Bowel symptoms and hydration	Have a written diarrhoea plan and emergency contact	Tell the nurse about cramps, sweating or diarrhoea during infusion	Delayed diarrhoea can start after the first day; act using your written plan	Fever, reduced urine, severe pain or inability to drink needs urgent advice	Restart decisions require recovery and team review
Fluorouracil safety and line	Confirm the dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) test and safe line/pump arrangements	Check mouth, bowel and pump symptoms early	Review line, pump and previous-cycle toxicity	Report chest pain or unexpectedly severe early side effects immediately	Confirm disconnection and ongoing line care
Cancer response and treatment choices	Record baseline disease and agree review plan	Bring forward review if symptoms worsen	Scans and other tests follow your consultant's treatment plan	Discuss benefit, side effects and whether the goal has changed	Agree continuation, a break, another treatment or supportive care
Conditional heart assessment	electrocardiogram (ECG) if cardiac function is compromised	Report new chest pain or breathing change	Emergency assessment for chest pain, collapse or severe breathlessness	Additional cardiac tests when clinically indicated	Follow any identified problem; no routine fixed echo series

Check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
international normalised ratio (blood-clotting test) (INR) if taking warfarin	Tell the team about warfarin	Check INR as clinically indicated	Report bleeding and major changes in food intake	Warfarin doses are adjusted by the prescribing team	Continue the individually agreed anticoagulation plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00585 v5b · current official regimen checked 5 November 2026 · EMA: Erbitux product information and patient leaflet

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Fortnightly cetuximab + FOLFIRI: treatment and checks

Advanced / metastatic colorectal cancer

Cetuximab and FOLFIRI start on Day 1, followed by a 46-hour fluorouracil infusion with planned disconnection around Day 3. Repeat every 14 days while beneficial and tolerable. No weekly Day 8 cetuximab is scheduled.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Blood count, kidney and liver NCCP 00585	Baseline blood count, kidney and liver profile	Review results and symptoms before treatment	Blood count, kidney and liver profile before every cycle; weekly blood counts when delayed for low counts	Repeat sooner if unwell or results need follow-up	Follow unresolved abnormalities after treatment
Magnesium, calcium and potassium NCCP 00585	Check mineral levels before starting	Repeat periodically during treatment	Keep blood-test and replacement appointments	More checks may be needed if levels fall or diarrhoea develops	Monthly checks for two months after the last antibody dose
Skin, nails, eyes and breathing NCCP 00585	Discuss existing skin, eye or lung problems	Start the prescribed skin-care plan; report reactions early	Review skin, nails, diarrhoea and new symptoms	Eye pain, worsening breathing or severe rash needs prompt assessment	Continue care until symptoms and mineral levels settle
Bowel symptoms and hydration NCCP 00585	Have a written diarrhoea plan and emergency contact	Tell the nurse about cramps, sweating or diarrhoea during infusion	Delayed diarrhoea can start after the first day; act using your written plan	Fever, reduced urine, severe pain or inability to drink needs urgent advice	Restart decisions require recovery and team review
Fluorouracil safety and line NCCP 00585	Confirm the DPD test and safe line/ pump arrangements	Check mouth, bowel and pump symptoms early	Review line, pump and previous-cycle toxicity	Report chest pain or unexpectedly severe early side effects immediately	Confirm disconnection and ongoing line care
Cancer response and treatment choices NCCP 00585	Record baseline disease and agree review plan	Bring forward review if symptoms worsen	Scans and other tests follow your consultant's treatment plan	Discuss benefit, side effects and whether the goal has changed	Agree continuation, a break, another treatment or supportive care

Care / check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Conditional heart assessment NCCP 00585	ECG if cardiac function is compromised	Report new chest pain or breathing change	Emergency assessment for chest pain, collapse or severe breathlessness	Additional cardiac tests when clinically indicated	Follow any identified problem; no routine fixed echo series
INR if taking warfarin NCCP 00585	Tell the team about warfarin	Check INR as clinically indicated	Report bleeding and major changes in food intake	Warfarin doses are adjusted by the prescribing team	Continue the individually agreed anticoagulation plan

What each check is for

1

A 14-day reference cycle

Clinic → home → review

The aim is to control advanced bowel cancer and symptoms. Treatment continues while it is helping and side effects remain acceptable; your team reviews the plan with you.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00585

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.35